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Correct Answer

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Practice Exam - 1

Question 32 of 117



A 38-year-old man is evaluated for management of nonischemic cardiomyopathy. He was diagnosed 6 months ago after presenting with exertional dyspnea. He is currently NYHA class II, compensated on physical examination and taking maximally tolerated quadruple guideline-directed medical therapy. ECG shows a RBBB at 125 msec. Transthoracic echocardiography shows a left ventricular ejection fraction of 42% with mild left ventricular dilation.

His history is notable a father who died suddenly at age 47. The patient's genetic testing reveals a pathogenic LMNA (lamin A/C) truncating variant. Cardiac MRI shows mid-myocardial late gadolinium enhancement involving the basal septum.

Which of the following is the most appropriate next step in management?

A. Continue current therapy with plans to reassess LVEF in 6 months to ensure it has not declined to <35%

B. Start oral amiodarone with a 5-gram load followed by 200 mg a day

C. Refer to EP for cardiac resynchronization therapy pacemaker (CRT-P)

D. Refer to EP for electrophysiology study to assess for inducible ventricular tachycardia to guide decision regarding need of implantable cardioverter-defibrillator (ICD).

E. Refer to EP for implantable cardioverter-defibrillator (ICD)

Commentary References

Correct Answer: E. Refer to EP for implantable cardioverter-defibrillator (ICD)

Teaching Point: The board-certified AHFTC specialist should recognize high-risk genetic cardiomyopathies such as LMNA-associated cardiomyopathy and understand when primary prevention ICD implantation is warranted even when LVEF is >35%.

The 2022 AHA/ACC/HFSA Management of HF guidelines provide a class 2A recommendation that “in patients with genetic arrhythmogenic cardiomyopathy with high-risk features of sudden death, with EF $\leq$ 45%, implantation of ICD is reasonable to decrease sudden death.”

Pathogenic truncating LMNA variants are such high-risk pathogenic variants. Risk factors for sudden death with LMNA/C include male sex, NSVT on ambulatory monitoring, LVEF<45%, non-missense mutations (e.g, insertion, deletion, truncating), and LGE on CMR.

Incorrect Answers:

1. ICD is currently warranted given the LMNA/C truncating variant and LVEF<45% in a man with family history of sudden death. You do not need to wait for LVEF to fall <35% to implant ICD.
2. Amiodarone is not adequate to mitigate the risk of sudden death in this setting
3. CRT-P is not currently indicated with LVEF 42% and RBBB of 125 msec.
4. Inducible VT by EP study is not needed to implant an ICD in this patient who already meets criteria for ICD implantation.

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